

Tranexamic acid (TXA) in trauma

LRI ED prescribing aid for adults

Do not use if known allergy to TXA or constituents,
or if significant thromboembolic disorder

Related guidance:

- UHL Adult Head Injury Guideline
- UHL Massive Haemorrhage Protocol

Patient details

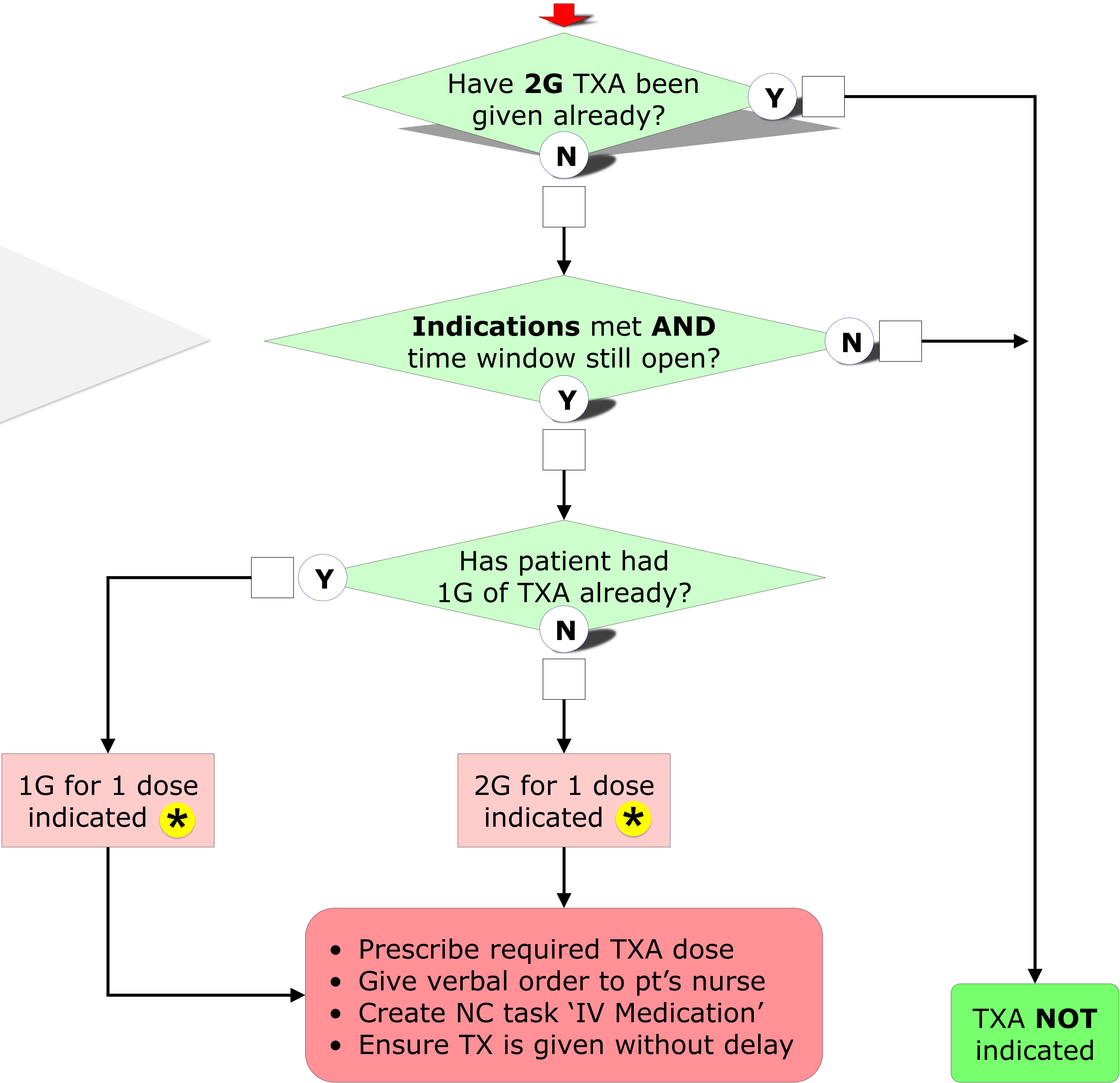
Full name

DoB

Unit number

(use sticker if available)

- Tick applicable indications
- ☐ Head injury with GCS on arrival 12 or less **AND** less than **2 hours** have passed since injury
- OR
- ☐ CT-positive head injury **AND** less than **2 hours** have passed since injury
- OR
- ☐ Less than **3 hours** have passed since injury **AND** patient shows evidence of significant bleeding, or risk of significant bleeding (at least one the below):
- ☐ Clinically hypovolaemic
- ☐ Heavy ongoing external bleeding (**NB**: Excludes compressible bleeding that has stopped)
- ☐ Suspicion of significant internal injury
- ☐ CT scan for potential bleeding planned
- ☐ Cross-match planned



Prescriber

Print name

Signature

Role

Date

Time completed

- DO NOT** give through same cannula while blood or other drug infusions are running, and flush afterwards
- To prescribe in NC Meds, go to Emergency Medicine (ED) > Bleeding/reversal meds (ED) > TXA (ED)
- During NC downtime, prescribe on UHL adult paper drug chart as shown below (see **ONE** for 1G , **TWO** for 2G)

PARENTERAL INFUSIONS							
Infusion Fluid		Additions to Infusion					
Date	Type/Strength	Vol.	Medicine	Dose	Route	Time to run or ml/hr	Prescriber
1 DD/MM/YY	NaCl 0.9%	100ml	Tranexamic acid	1G = 10ml	IV	660ml/h = over 10min	YOUR NAME & signature
2 DD/MM/YY	NaCl 0.9%	100ml	Tranexamic acid	2G = 20ml	IV	360ml/h = over 20min	YOUR NAME & signature